

Reporting to the board

The management interface: what escalates, what is reported, and how the Afya relationship is governed

The board has been trained. The instruments are drafted. On Thursday 24 September the board adopts them and runs its first meeting on the new model. Everything that board will read, question and decide on that day is written by the people in this room. This session is about that job: reporting by exception, escalating without being asked, and working inside the delegation of authority.

Dr Debo Odulana, Independent Non-Executive Director

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Why management is in a governance programme

Governance is often taught as something that happens to management. It is not. A board can only be as good as the information it is given, and that information is produced here.

WHAT THIS SESSION IS NOT

- It is not the board learning to check up on you.
- It is not more paperwork for its own sake. Three pages replace a long verbal update.
- It is not a transfer of blame. Escalation protects the person who escalates.
- It is not about Afya being distrusted. Structure is not suspicion.

WHAT IT IS

- The rules that tell you which decisions are yours, so you can take them without asking.
- A short, honest reporting format that gets you a decision instead of a debate.
- Thresholds that carry bad news upward automatically, so nobody has to judge the moment.
- A shared language with the board about what good reporting and good challenge look like.

A well-drawn governance system gives management more freedom, not less. Everything below the line is yours to decide, and the line is now written down.



What you will be able to do by the end

- 1. Prepare a report by exception against target on the Havana template, in three pages, and know what belongs in each section.
- 2. Answer the four standing questions on any variance before a director has to ask them.
- 3. Read the delegation of authority and say whether a decision is yours, the joint committee's, a board committee's, or the board's.
- 4. Name what escalates, to whom, and within what time, without waiting for the quarter and without waiting to be asked.
- 5. Describe how information flows from the ward and the department, through the Afya-HSH Governance Committee and the board committees, to the board.
- 6. Explain your own record-keeping and legal duties in supporting the board, and work to the calendar that binds the September cycle.

The test is not this session. The test is the pack that goes out on Thursday 17 September.



Where you sit, in one sentence

The board directs the hospital. Management runs it. The board holds management to account for running it, and it can only do that on what management tells it.

Three organs, from Module 1: the owners, the board, and management. Each has work the others cannot do.

THE OWNERS

Two founding families. They own the company, appoint the board, and decide the matters reserved to shareholders. They do not run the hospital, and they do not instruct you.

THE BOARD

Eight directors, meeting quarterly. Sets direction, oversees performance and risk, holds management to account, protects the owners and the mission.

MANAGEMENT

You, with the Afya executive. Runs the hospital every day, inside the budget, the policy and the delegated authority the board has set.

A single director, however senior, cannot instruct you. The board speaks through its decisions, minuted, and through the Chair.



The board's four jobs, and what each one asks of you

Everything the board wants from management traces back to one of these four. If a request does not, it is worth asking which job it serves.

THE JOB	WHAT IT MEANS	WHAT IT ASKS OF MANAGEMENT
Set direction	Strategy, the annual plan, the budget, the risk appetite.	Your plan, your forecast, and an honest view of what could knock it off course.
Oversee performance and risk	Against the plan the board approved, and the risks on the register.	The dashboard, on time, with actual against plan and prior period, and the risks you own.
Hold management to account	Constructively, on the evidence, not on personality.	Variance explained before it is asked about, and what is being done, by whom, by when.
Protect the owners and the mission	The licence, the patients, the reputation, the two families' asset.	Escalation the moment a threshold is crossed, whatever it looks like.



How information travels from the ward to the board

Four layers, each with a different frequency and a different job. Nothing skips a layer except an escalation, which is designed to skip all of them.

DAILY AND WEEKLY

Management and the departments

The hospital runs. Decisions inside budget, policy and delegated authority are taken here and not referred upward.

MONTHLY, FIRST TUESDAY

Afya-HSH Governance Committee

Operational performance against the agreed KPI set, partnership milestones, service issues, decisions taken under delegation, and what is being escalated.

TWO TO THREE WEEKS BEFORE THE BOARD

The five board committees

The detail is examined here: finance and operations, audit and risk, quality and clinical governance, people and remuneration, transformation and digital. Each reports to the board on one page.

QUARTERLY

The board

Reads the pack seven clear days ahead, questions the exceptions, takes the decisions reserved to it, and records them with their reasons.

The escalation schedule is the exception to all of this. It goes straight up, the same day or within hours, and it does not wait for a meeting.



Working backwards from Thursday 24 September

The board meets on Thursday 24 September and adopts the instrument suite. Everything before it is a lead time, and lead times are not negotiable once the board has adopted the calendar.

Tue 1 Sep	Afya-HSH Governance Committee, monthly	Your monthly report and KPI pack. First run on the new format.
Thu 3 Sep	Transformation and Digital Committee	Papers to the Company Secretary five working days ahead.
Tue 8 to Tue 15 Sep	Committee week: Finance and Operations, Audit and Risk, Quality and Clinical Governance, People and Remuneration	Your committee papers and the underlying detail. The committees examine here so the board does not have to.
Mon 14 Sep	Management report by exception due to the Company Secretary	Three pages. Signed off by the Medical Director with the Afya operational lead.
Thu 17 Sep	BOARD PACK CIRCULATED, seven clear days	Nothing is added after this date. A paper that misses the pack waits for the next meeting.
Thu 24 Sep	BOARD MEETING, adoption of the instrument suite	The board sets the thresholds you will then work to.

After September: six-weekly to December, then quarterly. The lead times never change. Committees two to three weeks before, pack seven clear days before.



Reading the delegation of authority

One schedule, one owner per decision. It exists so you can act without asking, and so that the few things you must not decide alone are written down rather than assumed.

The money ladder

A Naira figure alone tells you who decides. Bands are indicative until the board sets them on 24 September.

- **Band 1** up to N2m per item, inside approved budget. You decide.
- **Band 2** N2m to N10m. The Afya-HSH Governance Committee decides.
- **Band 3** N10m to N50m. The board decides.
- **Band 4** above N50m, or any reserved matter. The board, and the owners where marked.

The five rules that catch everything else

- **Residual.** Anything not expressly delegated is reserved to the board. Nothing falls through by omission.
- **No splitting.** Related transactions are added together for banding. A spend cannot be split to stay under a limit.
- **Within authority.** Outside budget or outside policy escalates, whatever the amount.
- **Dual control.** Above Band 1, two authorised signatories, and the approver is never the sole executor.
- **Emergency.** Where a decision cannot wait, the Medical Director acts on clinical emergencies and reports it for ratification at the earliest opportunity.

When in doubt, escalate. That is a rule in the schedule, not a personal preference. Nobody at Havana is criticised for putting something up a level.



What never comes to management

However sensible the answer is. Learn this list: it is short, and it is the single most common way a well-run hospital creates a governance problem for itself.

Reserved to the board

- Strategy, the annual business plan, and the budget.
- Opening or closing a service line or a facility.
- Capital projects at Band 3 and above.
- Appointment or removal of the Medical Director, the auditor, and the Company Secretary.
- Borrowing, guarantees, and security over assets.
- Risk appetite and the enterprise risk register.
- The annual operating plan and KPIs under the Afya agreement.

Reserved to the owners

- Changes to the constitution or the articles.
- Share capital, share issues, and shareholding structure.
- Dividends and distributions.
- Sale, merger, acquisition, or winding up.
- Any change to, or termination of, the Afya management agreement.
- Related-party transactions above Band 4.

Note the Afya agreement sits with the owners, not with the joint committee and not with management. Nothing in the partnership may be varied, waived, or informally agreed below that level.



Reporting by exception, and what it is not

This is the single biggest change to how you write for the board. Three pages, not thirty. Directors are there to exercise judgement, and judgement needs the exceptions, not the whole record.

Not this

- A narrative of everything that happened last quarter.
- Every metric listed, on plan and off plan alike.
- Good news first and at length, problems late and softened.
- A verbal update with no paper behind it.
- Data without a view. Numbers with nobody's judgement attached.

This

- Only what is off plan, and what is newly on the horizon.
- Each exception with its cause, its trend, its owner and its date.
- The uncomfortable item stated first, in your own words.
- Your recommendation, so the board decides rather than drafts.
- The full data available underneath, in the committees, on request.

The rule that makes the suite work: nothing reaches the board except through one of these instruments. A verbal update with no paper is not a board item.



The management report by exception, section by section

Three pages maximum. Written by the Medical Director with the Afya operational lead. Same six sections every quarter, so the board learns where to look and reads it in ten minutes.

- | | |
|--|--|
| 1 The three things the board should know
Three bullets. If a director reads nothing else, this is what they must not miss. At least one should be uncomfortable. | 2 Performance against plan
Reference the dashboard. Narrate only what is off plan, and answer the four standing questions for each exception. |
| 3 What went well, briefly
Two or three. Boards that only ever hear problems stop being told about them. | 4 What is coming
Next quarter: what you expect, and what could knock it off course. |
| 5 Where we need the board
Decisions requested, cross-referenced to board papers. Steers wanted. Anywhere management feels blocked. | 6 Declared escalations since the last meeting
What crossed a threshold, when, to whom, and how it was resolved. Nil returns are stated as nil. |

The Word template is instrument 05 in the pack. Do not rebuild it. Do not add sections. If something does not fit the six, it belongs in a committee paper.



"At least one should be uncomfortable"

A management report in which everything is fine is not reassuring to an experienced board. It is the thing that makes them start looking elsewhere.

Section 1 is three bullets, written last, in plain language. Two worked examples of the same quarter:

WEAK

- "Activity remained broadly in line with expectations during the period."
- "Some staffing challenges were experienced in certain areas."
- "Work is ongoing to address receivables."

No number, no owner, no date. The board must interrogate to learn anything.

STRONG

- "Theatre utilisation is 54 per cent against a plan of 70. Two lists a week are being lost to anaesthetic cover."
- "Night nursing has been below establishment for eleven of thirteen weeks. I judge this a patient safety risk, not a rota problem."
- "HMO receivables over 90 days have doubled to N83m. I need a board steer on suspending one payer."

The strong version is more exposing and far safer. A board that was told cannot later say it was not.



The four standing questions

Answer them before they are asked. Every director has been taught to put these to every variance, so answering them in the paper moves the meeting to the decision instead of the interrogation.

1

Why is it different from plan?

The cause, not the category. "Locum spend rose" is a category. "Two anaesthetists resigned in June and cover is agency" is a cause.

2

Is it one-off, or a trend?

Show the prior period and the direction of travel. The board treats a trend very differently from a one-off.

3

What is being done, by whom, by when?

A named owner and a date. Without both, it is a hope, and the board will treat it as one.

4

What would make it worse?

The honest downside. This is the question that most reports duck, and the one that most protects you when you answered it early.

Four sentences per exception. That is the whole discipline.



The honesty test of the whole system

If serious things happened in the quarter and section 6 is empty, the escalation thresholds are not being used. That is a governance failure whatever the underlying result looked like.

Section 6 records every threshold crossed since the last meeting: what it was, when it was escalated, to whom, and how it was resolved. Three things follow from that.

- **A nil return is stated as nil.** Silence is not a nil return. "No thresholds were crossed this quarter" is a sentence someone has signed.
- **The board reconciles it against what it already knows.** The Chair, the committee chairs and the Company Secretary receive escalations in real time. Section 6 is where those meet the record.
- **It is read as evidence the system works.** A quarter with four escalations, all handled, reads better to an experienced board than a quarter with none.

Write section 6 first, at the start of the quarter, and add to it as things happen. Reconstructing it from memory at the end is how items go missing.



What goes up immediately, to whom, and by when

These are the board's proposed defaults, to be set on 24 September. They exist so that nobody has to judge the moment. If the trigger is met, the clock starts.

TRIGGER	ESCALATES TO	TIMEFRAME
Never event, or a death in unexpected circumstances	Board Chair and Quality and Clinical Governance chair, then all directors	Same day
Serious clinical incident, or a cluster of related incidents	Medical Director to the Quality and Clinical Governance chair	24 hours
Regulatory inspection, notice, sanction, or licence issue	Board Chair and Audit and Risk chair	24 hours
Data or information security breach	Board Chair, Transformation chair, Audit and Risk chair, and the data protection lead	24 hours
Cash below the set number of days of operating cost	Board Chair and the Finance co-chairs	Immediate
Budget variance beyond the set threshold	Finance, Investment and Operations co-chairs	Next cycle, or immediate
Departure or suspension of a senior clinician or executive	Board Chair and the People and Remuneration chair	Same day
Loss of, or material change to, a significant HMO or corporate contract	Board Chair and the Finance co-chairs	48 hours
Any proposed change to the Afya agreement	The board, as a reserved matter	Before agreement

Also: litigation or a claim above the set threshold, and any risk newly rated 15 or above on the register. The full schedule is instrument 10.



Escalating is never penalised

This is the sentence to take away from the session, and the board will say it out loud when it adopts the schedule on 24 September.

A threshold crossed and escalated

is the system working. Nobody is criticised for it, and nobody is asked why they bothered. It is recorded in section 6 and read as evidence that the hospital notices things.

A threshold crossed and not escalated

is the only version of this that is a disciplinary matter. Not the underlying event. The failure to pass it on.

Which means an escalation is not an admission. It is the thing you were asked to do. The judgement the board makes is about the system, not about the messenger.

- **Escalate on the trigger, not on the outcome.** You do not wait to see whether it turns out badly. The trigger is the trigger.
- **Escalate up, not sideways.** To the named chair, copied to the Company Secretary, who records it. Not to the director you know best.
- **Escalating does not hand over the problem.** You still own the management response. The board owns knowing about it.



The partnership, and the one structural point

The Afya-HSH Governance Committee is jointly controlled with Afya. It therefore cannot be the board's only line of sight over Afya's own delivery.

This is not distrust of the partner. It is the same principle that stops any executive from being the sole assurance over their own work. Three consequences for how you operate.

The joint committee runs the partnership

Monthly, first Tuesday. Operational performance against the agreed KPI set, milestones, service issues, and decisions taken under delegation. It reports to the board on one page, in the same format as every committee.

The board keeps its own read

Audit and Risk takes an independent view of Afya's financial delivery, and Quality and Clinical Governance of the clinical side. Expect to be asked for the same numbers twice, by two bodies, for different purposes.

The agreement itself is untouchable below the owners

The operating plan and KPIs are approved by the board. Any change to the agreement is reserved to the shareholders. Nothing is varied by practice, by custom, or by a helpful email.

For the Afya leads in the room: you report in as operators, and the board assures the delivery separately. Both are normal, and both are in the agreement.



Being challenged well

The board's job is to hold management to account. Done properly it is uncomfortable and impersonal. Here is what to expect, and how the strongest executives handle it.

- **Challenge is about the paper, not about you.** A director pressing on a variance is discharging a legal duty of care. It is not a signal of lost confidence.
- **"I do not know, I will come back by Friday" is a strong answer.** A guess that turns out wrong costs far more credibility than a date. Then meet the date, through the Company Secretary, so it is on the action log.
- **Never take an instruction from a single director.** However senior, however reasonable. The board acts through minuted decisions and through the Chair. Route it, and disclose that you were approached.
- **The in-camera session is routine, not an event.** The non-executives sit alone for ten minutes at the end of every meeting, with no executives and no Afya present. It happens every time, from the first meeting, precisely so that it never means anything in particular.
- **Bring the recommendation, not only the problem.** Section 5 exists for this. A board asked to decide between your two options will decide. A board handed an open problem will discuss it and defer.

An executive who is never challenged is not being governed. If a quarter passes with no hard questions, something has gone wrong upstairs, not right downstairs.



Practice: what do you do, to whom, and by when?

Take these in pairs, five minutes, then we work them together. For each one: is this yours to decide, and does anything escalate, to whom, within what time?

1 The CT scanner fails on a Friday. Repair is quoted at N7.4 million and the vendor wants a decision today.

3 Night nursing has been below establishment for eleven of the last thirteen weeks.

5 HMO receivables over ninety days have doubled since the last quarter.

7 You need three additional beds' worth of equipment, N1.8 million, and it is in the approved budget.

2 A patient dies unexpectedly after a routine procedure. The next board meeting is in five weeks.

4 An Afya operational lead asks you to reallocate two theatre lists to a visiting surgeon on a revenue-share the agreement does not mention.

6 A director telephones you at home and asks you to expedite a relative's admission.

8 A staff member emails a spreadsheet of patient records to a personal account to work at home.

Answer with the instrument, not the instinct. Point to the band, the reserved matter, or the escalation trigger.



Six things, from September

The board adopts the suite on 24 September. These are the commitments that make it real on the management side, and the People and Remuneration Committee receives assurance that they are being met.

- **Use the templates as they are.** Instrument 5 for the board, the committee report for committees, the dashboard monthly. Do not rebuild them and do not add sections.
- **Hit the lead times.** Committee papers five working days ahead, the management report to the Company Secretary by Monday 14 September, and nothing added after the pack goes out on the 17th.
- **Answer the four standing questions in the paper.** Every exception, every time, with a named owner and a date.
- **Escalate on the trigger, the same day where the schedule says so.** To the named chair, copied to the Company Secretary. Then record it in section 6.
- **Keep the record.** Decisions taken under delegation, the risk register you own, and the registers the Company Secretary maintains. If it is not written down, it did not happen.
- **Tell the board what you need.** Section 5 is not decoration. A blocked executive who did not say so is a governance failure too.

The test of this programme is not that we all sat through it. It is that the meeting on 24 September runs on paper that was read, exceptions that were challenged, and decisions recorded with their reasons.

Questions.

Dr Debo Odulana, Independent Non-Executive Director. Consult for Africa.